

ADR Case Classification – Seriousness, Expectedness & Clinical Impact

Blue text = data entered from the case report (edit these). Black text = auto-calculated by formula. Cases are illustrative/

CASE DETAILS				
Case ID	Age / Sex	Suspect Drug	Indication	Adverse Drug Reaction
ADR-01	28 / F	Amoxicillin	Tonsillitis	Anaphylaxis (hypotension, bronchospasm) within 20 min of 1st dose
ADR-02	55 / M	Diclofenac	Osteoarthritis knee	Upper GI bleed (melaena, Hb fall 3 g/dL)
ADR-03	42 / F	Metformin	Type 2 diabetes	Nausea and loose stools in first 2 weeks
ADR-04	34 / M	Carbamazepine	Epilepsy	Stevens-Johnson syndrome (mucosal erosions, target lesions)
ADR-05	68 / M	Metformin	Type 2 diabetes with CKD	Lactic acidosis
ADR-06	46 / F	Pantoprazole	GERD	DRESS (rash, eosinophilia, hepatitis, fever) after 5 weeks
ADR-07	60 / M	Losartan	Hypertension	Persistent hiccups
ADR-08	50 / M	Atorvastatin	Dyslipidaemia	Rhabdomyolysis (CK >10x ULN, myoglobinuria)
ADR-09	62 / M	Ciprofloxacin	UTI	Achilles tendon rupture

ADR-10	30 / F	Sodium valproate	Epilepsy (pregnant)	Neural tube defect in newborn
ADR-11	25 / F	Cetirizine	Allergic rhinitis	Drowsiness
ADR-12	72 / M	Amlodipine	Hypertension	Agranulocytosis with sepsis



educational; 'Listed in RSI?' is judged against an ASSUMED reference label – verify against the actual current label/Smf

Outcome	ICH E2A SERIOUSNESS CRITERIA (Y/N)							S
	Death	Life-threatening	Hospitalisation	Disability / Incapacity	Congenital Anomaly	Medically Important Event	Seriousness	
Recovered	N	Y	Y	N	N	N	Serious	
Recovered	N	N	Y	N	N	N	Serious	
Recovered	N	N	N	N	N	N	Non-serious	
Recovering	N	Y	Y	N	N	N	Serious	
Fatal	Y	Y	Y	N	N	N	Serious	
Recovering	N	N	Y	N	N	N	Serious	
Recovered	N	N	N	N	N	N	Non-serious	
Recovered	N	N	Y	N	N	Y	Serious	
Recovering (residual weakness)	N	N	N	Y	N	N	Serious	

Not recovered	N	N	N	N	Y	N	Serious
Recovered	N	N	N	N	N	N	Non-serious
Fatal	Y	Y	Y	N	N	N	Serious

PC before real reporting.

SERIOUSNESS		EXPECTEDNESS		SEVERITY & CAUSALITY	
Criteria Met	Listed in RSI / Label?	Expectedness	Severity (intensity)	WHO-UMC Causality	
LifeThreatening, Hospitalisation	Yes	Expected	Severe	Probable	
Hospitalisation	Yes	Expected	Severe	Probable	
-	Yes	Expected	Mild	Probable	
LifeThreatening, Hospitalisation	Yes	Expected	Severe	Probable	
Death, LifeThreatening, Hospitalisation	Yes	Expected	Severe	Possible	
Hospitalisation	No	Unexpected	Severe	Possible	
-	No	Unexpected	Mild	Possible	
Hospitalisation, MedicallyImportant	Yes	Expected	Severe	Probable	
Disability	Yes	Expected	Severe	Probable	

Congenital	Yes	Expected	Severe	Probable
-	Yes	Expected	Mild	Certain
Death, LifeThreatening, Hospitalisation	No	Unexpected	Severe	Possible



REGULATORY CATEGORY			
Case Category	Reporting Timeline	Clinical Impact Level	
Serious Expected	15 calendar days	Critical	
Serious Expected	15 calendar days	Major	
Non-serious Expected	Periodic (PSUR / PBRER)	Minor	
Serious Expected	15 calendar days	Critical	
Serious Expected	15 calendar days	Critical	
Serious Unexpected	15 calendar days	Major	
Non-serious Unexpected	Periodic (PSUR / PBRER)	Minor	
Serious Expected	15 calendar days	Major	
Serious Expected	15 calendar days	Major	

Serious Expected	15 calendar days	Major
Non-serious Expected	Periodic (PSUR / PBRER)	Minor
Serious Unexpected	7 days (fatal / life-threatening SUSAR)	Critical



CLINICAL IMPACT
Clinical Impact – Interpretation
Immediate-type hypersensitivity; needs emergency care. Beta-lactam allergy to be documented.
Serious but well-known NSAID risk; older age is a risk factor. Preventable with gastroprotection.
Minor, self-limiting; may affect adherence if not counselled.
Life-threatening cutaneous reaction; HLA-B*1502 association in Asian populations.
Fatal outcome; renal impairment is a contraindication (label warning). Highlights need for eGFR monitoring.
Serious and NOT in assumed label – potential new signal; needs case follow-up and literature search.
Non-serious, unlisted (assumed); low impact but adds to signal detection if repeated.
Serious, listed class effect; risk raised by interacting drugs (e.g., macrolides).
Persistent incapacity to walk; fluoroquinolone boxed warning; risk higher with age/steroids.

Congenital anomaly – serious by definition; teratogenicity is a labelled contraindication in pregnancy.

Minor, dose-related sedation; counsel on driving and machinery.

Fatal, unlisted (assumed) reaction – priority potential signal requiring expedited reporting.



Action / Management
Drug stopped; IM adrenaline, IV fluids, steroids; penicillin allergy card issued
NSAID withdrawn; PPI + transfusion; safer analgesic substituted
Continue; take with meals; dose titrated slowly
Drug stopped permanently; burn-unit care; genotype screening advised for relatives
Drug stopped; ICU/haemodialysis; patient died – RCA on eGFR monitoring
Drug stopped; systemic steroids; report to PvPI; monitor for further cases
Dose reduced; symptoms resolved (positive dechallenge)
Statin stopped; IV hydration; re-challenge avoided
Drug stopped; surgical repair and physiotherapy

Counselling; folic acid; switch to safer antiepileptic pre-conception

Take at bedtime; no change of therapy

Drug stopped; G-CSF, IV antibiotics; patient died; expedited report filed

Summary Dashboard (auto-calculated from 'Case Classification')

Overview

Total cases evaluated	12
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Seriousness

Category	No. of Cases	% of Cases
Serious	9	75.0%
Non-serious	3	25.0%

Expectedness

Category	No. of Cases	% of Cases
Expected (listed)	9	75.0%
Unexpected (unlisted)	3	25.0%

Case Category (Seriousness × Expectedness)

Category	No. of Cases	% of Cases
Serious Unexpected	2	16.7%
Serious Expected	7	58.3%
Non-serious Unexpected	1	8.3%
Non-serious Expected	2	16.7%

WHO-UMC Causality

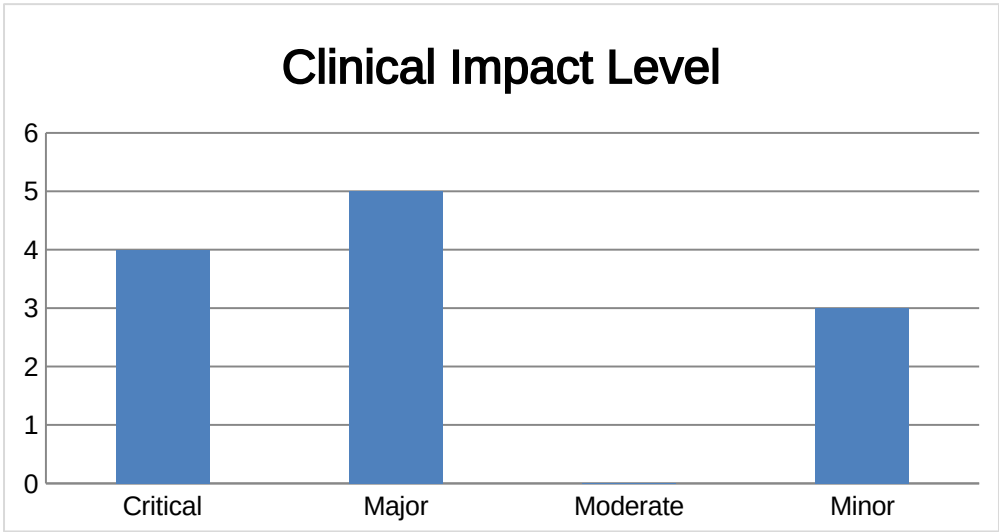
Category	No. of Cases	% of Cases
Certain	1	8.3%
Probable	7	58.3%
Possible	4	33.3%
Unlikely	0	0.0%
Conditional	0	0.0%
Unassessable	0	0.0%

Clinical Impact Level

Category	No. of Cases	% of Cases
Critical	4	33.3%
Major	5	41.7%
Moderate	0	0.0%
Minor	3	25.0%

Reporting Timeline

Category	No. of Cases	% of Cases
7 days (fatal / life-threatening SUSAR)	1	8.3%
15 calendar days	8	66.7%
Periodic (PSUR / PBRER)	3	25.0%



Pharmacovigilance Reference

1. Seriousness criteria (ICH E2A) – any C

Criterion
Death
Life-threatening
Hospitalisation
Disability / incapacity
Congenital anomaly / birth defect
Medically important event

2. Expectedness (ICH E2A / E2D)

Term
Expected (listed)
Unexpected (unlisted)
Seriousness ≠ Severity

3. WHO-UMC causality categories

Category
Certain
Probable
Possible
Unlikely
Conditional / Unclassified
Unassessable / Unclassifiable

4. Case category → action (as used in thi

Category
Serious Unexpected
Serious Expected
Non-serious Unexpected
Non-serious Expected

5. Clinical impact level (formula logic)

Level
Critical
Major
Moderate
Minor

*Note: Reporting timelines follow ICH E2A/E2L
are hypothetical for study purposes.*

Key

ONE makes the ADR 'Serious'

Meaning

- Reaction results in death
- Patient at immediate risk of death at the time of the event (not hypothetical)
- Requires inpatient admission or prolongs existing stay
- Persistent or significant disruption of ability to carry out normal life functions
- Malformation in offspring after exposure
- Judged serious by clinical judgement even if none of the above are met

Definition

- Nature/severity consistent with the Reference Safety Information (product label / SmPC / Investigator's Brochure)
- Specificity or severity not consistent with the RSI – triggers closer review as a potential signal
- Severity = intensity (mild/moderate/severe). Seriousness = outcome/regulatory criteria. Severe headache is not automatically serious; a mild-looking event causing hospitalisation is.

Key features

- Plausible time relationship; cannot be explained by disease/drugs; positive dechallenge; rechallenge satisfactory if necessary
- Reasonable time relationship; unlikely due to other causes; positive dechallenge; rechallenge not required
- Reasonable time relationship but could be explained by disease/other drugs; dechallenge information lacking or unclear
- Time relationship improbable; other drugs/disease provide plausible explanation
- More data needed for proper assessment
- Insufficient or contradictory information that cannot be supplemented

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Typical action

- Priority case: expedited report, signal evaluation, possible label change
- Expedited individual case report; monitor frequency
- Record; watch for clustering/signal
- Routine recording and patient counselling

Rule

- Death or life-threatening
- Serious (hospitalisation, disability, congenital anomaly, medically important) but not fatal/life-threatening
- Non-serious with moderate severity
- Non-serious, mild

3 conventions (7/15-day rule). Local rules (e.g., India PvPI / CDSCO, NDCT Rules 2019 for trials) may differ



Example
Fatal lactic acidosis
Anaphylaxis, SJS
GI bleed needing admission
Tendon rupture
Valproate – neural tube defect
Rhabdomyolysis, blood dyscrasia



Reference
Label / RSI
Label / RSI
ICH E2A





Timeline used
7 days if fatal/life-threatening; else 15 days
15 calendar days
Periodic (PSUR/PBRER)
Periodic (PSUR/PBRER)



· – *always confirm current regulatory requirements. Cases*